

JOHNS HOPKINS MEDICINE

Neurology Referral Intake (Fax Received)

Fax timestamp: 02/14/2026 16:22 EST

Pages received: 11 (per machine)

Referral reason: Tremor / ?Essential tremor vs Parkinsonian features

Requested service: General Neurology (Movement?)

Priority: Routine

Referring provider: Dr. Alan Weiss, MD (Internal Medicine)

Phone: (703) 555-8821 Fax: (703) 555-7710

Comments:

“Pt with worsening tremor not responding well to propranolol. Some stiffness maybe. Family worried. Please eval.”

Insurance: BCBS PPO (card illegible)

Patient aware of referral: yes (per front desk note)

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PATIENT DEMOGRAPHICS (print view)

Name: Robert J. Kline

DOB: 03/18/1958

Age: 67

Sex: M

Address: 1427 Willow Bend Dr, Centreville, VA 20120

Phone: 571-555-2399 (home), 571-555-9811 (cell – sometimes off)

Marital Status: Married

Emergency Contact: Susan Kline (wife) 571-555-1182

Primary Care Provider: Alan Weiss, MD

Preferred Pharmacy: CVS #1847 Centreville

Allergies: NKDA (one note says “possible sulfa rash?” unclear)

Height: 5’10”

Weight: 184 lb (last recorded 1/28/26)

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REFERRING PROVIDER NOTE (Office Visit 01/28/2026)

Chief Complaint: Tremor follow up

History of Present Illness:

67 y/o male with approx 2–3 year history of tremor, initially noted in R hand while writing and

holding utensils. Pt reports progression, now sometimes at rest “when watching TV” per wife, though pt states mostly when using hand. Tremor described as “pill-rolling?” (wife used this term). Has tried propranolol 20 mg BID with minimal improvement, dose increased to 40 mg BID, still limited effect.

Also reports mild stiffness in R arm, slower getting dressed, some decreased arm swing? Denies falls. Denies significant gait issues though wife thinks he “shuffles sometimes.” No dizziness.

Denies alcohol improving tremor (uncertain – pt says he doesn’t drink much). No clear family history of tremor.

Sleep: wife reports “acting out dreams” occasionally (unclear frequency).

Review of Systems:

- constipation (chronic)
- decreased sense of smell (longstanding per pt, “years”)
- headache
- vision loss

Past Medical History:

HTN

Hyperlipidemia

GERD

“Essential tremor?” (added 2024)

Assessment:

Tremor – unclear etiology. Some features atypical for ET. Consider neuro referral.

Plan:

Continue propranolol for now

Refer neurology

Consider trial primidone if not seen soon

Electronically signed 01/28/26 14:02

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PRIOR NOTE #1 (Annual Physical 05/14/2025)

Chief Complaint: Annual exam

HPI:

Pt notes mild tremor in right hand for “about a year,” mostly when drinking coffee or using tools.

No resting tremor reported at that time. No rigidity noted. No gait complaints. Family history: father had “shaky hands in old age” (unclear dx).

Alcohol reportedly helps slightly (pt says “a beer calms it down”).

Exam:

Neuro: mild postural tremor R hand. No cogwheeling documented. Gait normal.

Assessment:

Likely essential tremor.

Plan:

Start propranolol 10 mg BID

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PRIOR NOTE #2 (Urgent Care Visit 11/02/2025)

Reason for Visit: Right arm stiffness, shoulder pain

HPI:

Pt presents with R shoulder discomfort and “tightness” for several months. Reports difficulty swinging arm while walking. Also mentions tremor worse lately. Denies injury.

Exam:

MSK: reduced arm swing noted while ambulating

Neuro: tremor present R hand, unclear if rest vs action (not documented clearly). Tone “slightly increased?”

Assessment:

Shoulder strain vs cervical radiculopathy

Tremor – ongoing

Plan:

NSAIDs

Physical therapy referral

Follow up PCP

(Note: no imaging done)

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PRIOR NOTE #3 (Follow-up 12/15/2025)

Chief Complaint: Tremor worse

HPI:

Pt states tremor now interfering with eating. Wife reports seeing tremor when patient's hand is resting in lap. Pt unsure. No falls. Some slowness with buttons.

Denies alcohol effect (contradicts prior note).

Exam:

Neuro: resting tremor R hand observed briefly. Mild rigidity? Not clearly documented. Finger tapping slightly slow on right.

Assessment:

Essential tremor vs other movement disorder

Plan:

Increase propranolol

Neurology referral discussed (not yet placed)

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LAB RESULTS (selected)

CMP 01/28/2026

Na 139

K 4.2

Creatinine 1.0

AST 22

ALT 19

CBC

WBC 6.2

Hgb 14.8

Platelets 240

TSH 1.82 (normal)

B12 412

Lipid panel (irrelevant)

LDL 132

HDL 41

A1c 5.8%

PSA 2.1

Urinalysis (routine) negative

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IMAGING REPORT

MRI Brain without contrast

Date: 01/10/2026

Indication: Tremor, rule out structural lesion

Findings:

No acute infarct or hemorrhage.

Mild generalized volume loss consistent with age.

No focal lesions.

Basal ganglia grossly unremarkable.

Impression:

No acute intracranial abnormality.

Radiologist: K. Simmons, MD

(Note: image quality “some motion artifact”)

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MEDICATION LIST (reconciled poorly)

Propranolol 40 mg BID

Lisinopril 20 mg daily

Atorvastatin 20 mg nightly

Omeprazole 20 mg daily

Aspirin 81 mg daily

Vitamin D 1000 IU daily

Multivitamin

Melatonin PRN

Ibuprofen PRN

Primidone 50 mg nightly (started? patient unsure if taking regularly)

Old meds still listed:

Metoprolol (discontinued 2023?)

Simvastatin (duplicate)

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ADDITIONAL DOCUMENT 1 (Physical Therapy INITIAL EVAL 11/20/2025)

Reason: Shoulder stiffness

Subjective:

Pt reports difficulty swinging right arm while walking. Therapist notes decreased reciprocal arm swing and mild trunk stiffness.

Objective:

Gait: slightly reduced arm swing R

Balance: normal

Strength: 5/5

Assessment:

Possible neurological component? Recommended MD follow-up

Plan:

Therapy 2x/week

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ADDITIONAL DOCUMENT 2 (Sleep Clinic Intake 08/2025)

Reason: Snoring / sleep disturbance

HPI:

Wife reports patient "kicks and yells in sleep sometimes." Pt unaware.

Assessment:

Possible REM behavior disorder vs OSA

Plan:

Sleep study recommended (not completed)

Other notes:

Patient mentions reduced smell "for years"

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